THE PROBLEM · 1 OF 2 PARTS

The UK doesn't have one patient record.

It has several — and the thing joining them is not a database.
It's **letters**.

Swipe → the problem, then what can be done about it

Four records, four owners

GP practice

Independent contractors, not NHS-owned. The lifelong record sits in EMIS Web or SystmOne.

Hospital trust

Runs its own electronic patient record. Separate system, separate login.

Pharmacy & community

Their own dispensing and care records, often with no direct link to the practice.

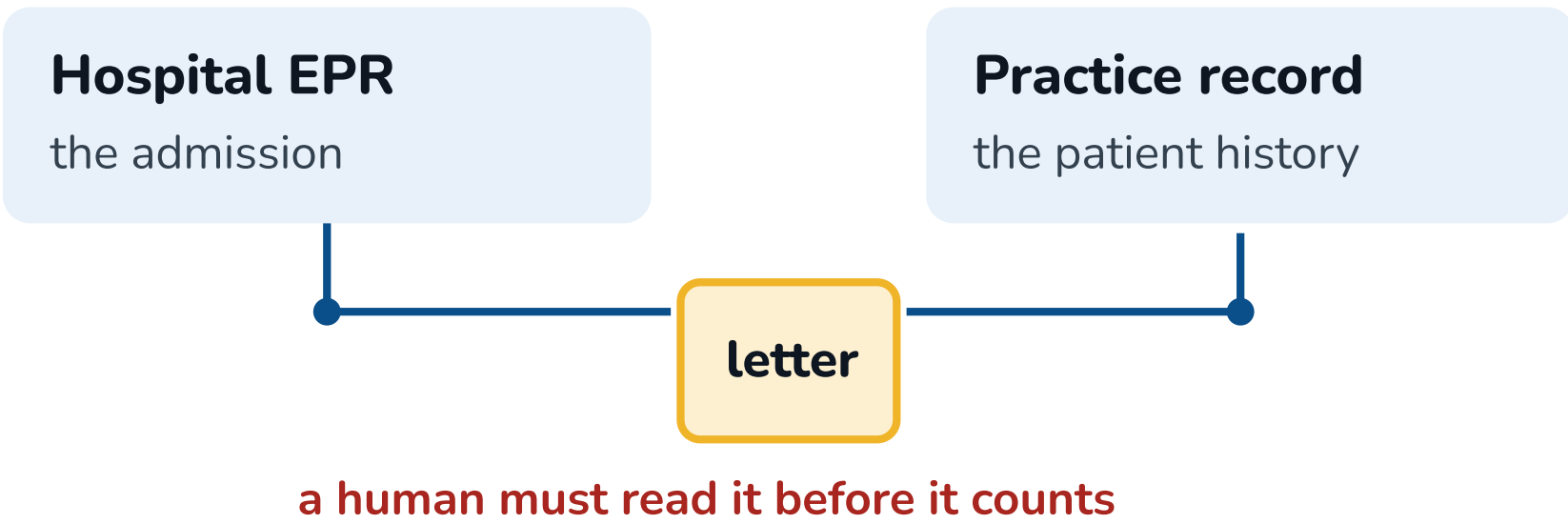
National services

NHS number, Spine, GP Connect, e-RS — the shared plumbing, not the record itself.

THE PROBLEM · THE JOIN

So what connects them?

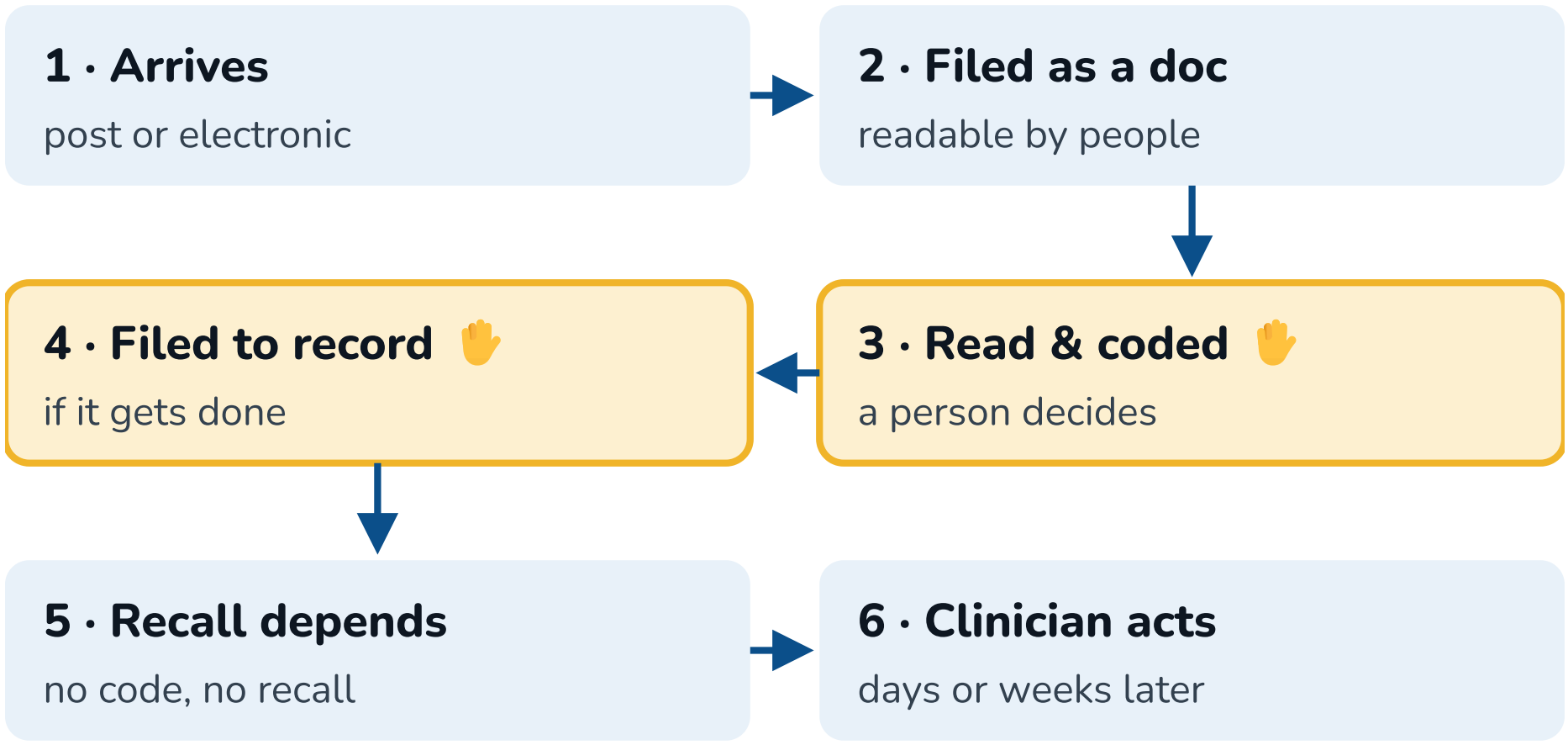
Discharge summaries and clinic letters, arriving as documents. A PDF is not a database row.



The practice doesn't receive data. It receives **a document about data**.

THE PROBLEM · THE MANUAL STEP

Two of six steps are done by hand



Steps 3 and 4 are the gap. The letter has arrived — the information has not.

THE PROBLEM · THE CONSEQUENCES

What the gap produces

- **Re-reading** — read once to understand it, again if it was coded wrong.
- **Coding lag** — the fact exists on paper but not in the record being searched.
- **Recall misses** — a recall query pulls by code; free text is invisible to it.
- **Duplication** — the same datum typed into a second system, because they can't talk.
- **Delay** — a result in an inbox is clinically the same as one that never arrived.

THE SOLUTION · 2 OF 2 PARTS

Fix the manual middle.

Not another record system. A way to turn the document into a **coded, confirmed fact** — with the clinician still holding the decision.

This is what **HealthBridge** does.

THE SOLUTION · HOW IT WORKS

Four steps, one of them human

1 · Read

Take the letter or PDF. Extract the diagnosis, the medicine, the action, the dates.

2 · Propose

Suggest a SNOMED CT code for each fact, with a confidence — not a decision.

3 · Confirm (clinician)

Accept, edit or reject. Nothing enters the record before this step.

4 · File & export

Write FHIR R4 records on the NHS number, with an audit trail of who did what.

THE SOLUTION · WHAT IT SPEAKS

Built on the standards already in use

STANDARD	HEALTHBRIDGE USE
NHS number	Links every record to the right person
SNOMED CT	The proposed clinical code, SCCI0034 terminology
dm+d	One medicine, one code, prescribing to dispensing
FHIR R4	Records written and read back as data, not documents
Audit log	Who read it, who coded it, who changed it

Interoperation, not replacement — the practice keeps its own record.

THE SOLUTION · WHAT IT WILL NOT DO

Three refusals

- **No autonomous diagnosis.** The model reads; a clinician decides.
- **No shared patient pool.** One practice cannot see another's patients.
- **No silent writes.** Every filed fact is attributable and reversible.

And an honest limit: HealthBridge holds **no UK certifications yet** — DTAC, DSPT, DCB0129 and a DPIA are what a UK deployment would require, and slide-level claims don't substitute for them.

SEE IT

Six screens, thirty seconds.

The clinic loop, clickable — registration, vitals, consultation, prescription, pharmacy, follow-up. Sample data, no sign-up.

19p1362.github.io/portfolio/healthbridge

Gonela Abhilash · CPC · registered pharmacist · MSc Public Health, ARU

Independent project. Not affiliated with the NHS. No compliance claim is made.